QUICK CARD · EMERGENCY CALL-TAKING

The first 60 seconds

Triage the patient, not the panic.

1 Location
Get the address of the emergency and **read it back**. Unit number, cross street, landmark.
"What's the address of the emergency?"

2 Callback number
If the line drops, it's your only way back to the caller.
"What number are you calling from?"

3 Exactly what happened
An open question gets the chief complaint fast. Listen for key words.
"Tell me exactly what happened."

4 The big two
Conscious? Breathing normally? These two answers drive the priority.
"Is he awake? Is his breathing normal, or is it gasping?"

5 Code, then coach
Set the priority so the right crew moves, then give instructions.
"I'm going to tell you exactly what to do."

PRESSURE TRAPS

- **"He's snoring."** In a collapsed, unresponsive patient, snoring or gasping can be agonal breathing. Treat it as **not breathing normally**.
- **"She's stopped fitting."** Stopping isn't safe. Re-check response and breathing.
- **"It's just indigestion."** Callers downplay. Chest pain with sweating or pallor is a red flag.
- **A screaming caller.** Volume isn't severity. Give one clear task in a calm, firm voice.

Portfolio sample by Shammim · Triage Under Pressure micro-learning. Simplified for learning: it is not the operational protocol of Triple Zero Victoria or any emergency service. Always follow your service's structured call-taking protocol.